

I: So then we will move on to the first question.

I: This is about familiarity with AI.

I: How familiar are you with AI technologies in general?

P: Am I supposed to give you a number?

I: These are open questions.

I: For some of them, I also want a number.

I: For this one as well.

I: First answer openly, and then I'll ask the score.

P: All right.

P: I'd say pretty much, not top level, but I very often use ChatGPT and Gemini.

P: Both in my personal and professional life.

I: Okay, so what would you say is your score of familiarity, one is low and ten is high?

P: Seven.

I: Okay.

I: You mentioned you use ChatGPT and Gemini in your personal life and also at work.

I: How have you used it in both cases?

P: In my personal life, whenever I have a question, I just ask ChatGPT straight away.

P: It's easier than using the internet or Google.

P: Professionally, whenever I have a question about tests, like how to apply a test or how to understand a score, I use it.

P: First I try to do it myself, and then if I need to check, I use ChatGPT.

P: I've also seen colleagues use it to write reports and documents like that.

I: So it's mostly to confirm how tests are administered and how to interpret results.

P: Yeah.

I: Has your organization introduced any AI tools or systems?

I: You use it at work, but was that your own decision or something introduced by the organization?

P: It's not mandatory.

P: My colleagues started using it first to make sure they were doing things right.

P: Then our bosses paid for Gemini Pro.

P: So now it's another resource we have.

I: And do you disclose AI use to clients?

P: Sometimes.

P: For example, if we use ChatGPT to generate a text to work on attention exercises, we might say it.

P: But not for reports or formal documents.

I: So do you not use it for reports, or do you just not disclose it?

P: We do not disclose it.

P: I haven't used it for reports myself, but I know colleagues who do and don't mention it.

I: Okay.

I: So then we can move on to the next question.

I: This is about diagnostics and assessment use.

I: What are your thoughts on AI being used in diagnostic or assessment processes?

P: I think first we need to be aware of how tests are administered and how to interpret results.

P: We need to be professionals and deal with things ourselves.

P: I would only use AI to make sure my hypotheses are correct or to contrast information.

P: I wouldn't rely on it completely.

I: So you wouldn't blindly follow AI advice.

P: Exactly.

I: If AI wasn't there, would you do this checking with colleagues?

P: Yes, with my colleagues.

I: Do you do that less now because AI is available?

P: Not really.

P: Sometimes we discuss things together.

P: And if we still have doubts, then we check.

I: I'm not completely sure about your role.

I: You do rehab, but do you also have intake sessions like in therapy settings?

P: It depends on the patient.

I: Can you explain the patient pathway?

P: First they come to the center and explain what they've been through.

P: Then there is a screening for physiotherapy, neuropsychology, occupational therapy, and speech therapy.

P: According to their deficits and strengths, the team reaches a consensus on what the rehab process should be.

P: If neuropsychology is needed, which is often the case, they usually have intensive training.

P: The first two years are very important.

P: They might have three sessions a week.

P: First we do a formal and thorough assessment of attention, memory, executive functions, comprehension, and so on.

P: Then there are different rooms for each area.

P: When they come, they do exercises to strengthen the abilities they've lost.

I: Okay.

I: That makes sense.

I: Have you considered AI for preliminary screening?

I: Are there parts of that process that could be done by AI?

P: I don't think so.

I: Okay.

I: And what about summarizing patient information?

P: Not so much for summarizing.

P: But it is useful for understanding medical information, because we're not doctors.

P: Especially for patient history.

I: Yeah, because most of these clients will have some medical underlying problems which are causing the mental problems, right?

I: Yeah.

I: Like the neural, yeah.

I: Um, okay, then I think we can move on.

I: This is for monitoring and progress tracking.

I: How do you feel about the role of AI in tracking patient progress over time?

I: These questions are based on a more typical psychotherapy setting, so they might not always fully align with what you do.

I: We'll go through them and look at what might be beneficial for you.

I: For monitoring progress, you said it's much more intensive than most psychotherapy, right?

I: You do three sessions a week.

I: And in most of these cases there are injuries, and the first two years are crucial for recovery.

I: So do you do any monitoring and progress tracking?

P: We do.

P: Usually every three or six months.

P: That depends on the injury and the case itself.

P: We administer tests twice.

P: There's a first test, then after three to six months we do it again and check the differences.

P: I wouldn't say AI is necessary here, but sometimes if something doesn't make sense, for example if someone has improved in memory but not in attention, or the results are even lower than at the beginning, then maybe AI could help us understand that progress.

P: Usually they either improve or stay the same.

P: So yeah.

I: Yeah.

I: So after those three to six months, are they done?

I: Is that like a final test?

P: No.

P: We often do follow-ups until we see there is no more improvement.

P: That means the person can probably no longer recover their abilities.

P: After every follow-up we have a meeting with the person and their family.

P: We give them all the information we have and adjust the number of sessions or what we're doing if needed.

P: When we see there is no more improvement, then the person usually leaves.

I: Yeah.

I: Okay, makes sense.

I: Is there a basic time frame per patient, or is it very patient dependent?

I: Like when someone doesn't improve anymore, that's probably the limit of their recovery.

P: Usually it's around a year and a half to two years.

P: It's not fixed, it's patient dependent.

P: Also, if someone has suffered an accident and comes to us a year later, there's not much we can do.

P: The sooner you start, the better.

P: Some patients recover for a longer period, some for shorter.

P: It depends.

I: Yeah.

I: And AI wouldn't really be helpful here, right?

P: Yeah.

I: Maybe only for understanding something incoherent.

I: Okay.

I: Then we can move on to the next question.

I: This is about administrative and practical support.

I: How do you feel about AI generating drafts or reports or assisting with scheduling and reminders?

P: I've never used it for reminders or scheduling.

P: I suppose it could be useful, but I just turn on my alarms, which is very fast.

P: So yeah.

P: What else did you ask?

P: Reports.

I: Yeah, drafts or reports.

P: I don't know.

P: I like stimulating myself and not just leaving it to the AI.

P: I do my things first and then just check.

I: So you mostly see a use case for checking if everything is correct or if something doesn't make sense?

P: Yeah.

P: I'm sure that if I enter the data and ask AI, the results and explanations will be fine.

P: But I want to do it myself.

P: There is also cognitive data that is more implicit in sessions.

P: So there's this human factor.

I: What type of data would that be?

P: For example, we sometimes start sessions in the neuropsychology room.

P: But there are also common areas where physiotherapists and occupational therapists work.

P: If we're working on selective attention, we might move the person to a common area to stimulate that.

P: Sometimes we see improvement, but there's no specific test that assesses it.

P: That's more a clinical impression.

P: We also test with formal measures, but in the sessions there's something more implicit.

I: So you might notice improvement in behavior or memory outside testing moments.

P: Yeah.

I: Okay.

I: Then we can move on.

I: The next question is about psychoeducation and between-session tools.

I: I'm not sure if this fully applies, but what are your thoughts on using AI for psychoeducation or therapeutic exercises between sessions?

P: I've used it for both.

P: For psychoeducation, mainly to outline what I can say clearly, because our vocabulary can be very technical and we need to adapt it to the patient's abilities.

P: That's very helpful.

P: Also for Instagram, we have an account with psychoeducation posts.

P: It helps organize ideas and decide what information goes on each slide.

P: For exercises, we use it a lot to create texts.

P: For example, when working on attention, we use a text and ask the person to cancel specific letters.

P: Creating those texts ourselves takes time.

P: So we ask AI to prepare them.

P: We also use it to create exercises or get ideas for emotion regulation.

P: Yeah.

I: So you already have specific tools, and instead of creating texts or homework yourself, you ask AI to generate them.

P: Yeah.

I: This might not fully apply, but what if AI provides information that contradicts your treatment approach?

I: For example in psychoeducation or homework.

P: I trust myself.

P: If I think the AI is not right, I would tell the AI that it's not right and see how it reacts.

I: Yeah.

I: Okay.

I: So do you think the AI use will only be used by you as the caregiver, or also by the patient?

P: No, I don't think the patient will use it.

I: Yeah, okay, makes sense.

I: For this question it was mostly focused on whether there would be an AI that supports your clients in between sessions, so that they would be interacting with AI themselves, not just you using AI to give something to the patient.

I: Okay, that makes sense, so this doesn't really apply.

I: So you would just use it yourself and confirm whether something is correct, or check if maybe you're wrong.

I: That's kind of the gist of it, right?

P: Yeah.

I: Okay, then we can move on to the next question.

I: This is about the risks and limitations.

I: What risks or concerns do you have about using AI in clinical work?

P: Personal information, mostly.

P: Because I've seen some colleagues use AI, probably ChatGPT, to ask about specific exercises or results.

P: But they entered personal data.

P: And I think that's a very big problem.

P: Every time I use AI to ask something, I delete any personal data that might be in the text.

P: I've seen that some colleagues haven't, maybe because they forgot or for other reasons or because they don't find it important.

P: That scares me, because it's happened to me in my personal life.

P: I've asked ChatGPT to give me an answer on something, and then it appeared like it had more information than what I had asked.

P: That might be because of other conversations I've had with it.

P: But it's not supposed to keep that information and then use it in a different chat or conversation.

P: I don't know.

P: I don't know.

P: I'm doubtful.

I: Yeah.

P: I don't think they're doing things exactly as they say they are.

I: Mm-hmm.

I: Any other risks or limitations?

P: Limitations like transference or countertransference in therapy.

P: When a patient makes you feel a certain way, or reacts in a specific way, and feelings arise.

P: ChatGPT or Gemini cannot feel that.

P: And I think that's very valuable in therapy.

P: Especially in psychotherapy.

P: Mainly, actually.

P: I don't know.

I: Yeah, have you thought about issues like bias?

P: What do you mean exactly?

I: Well, have you considered that AI might be biased in their responses or how it works?

P: Okay.

P: Yeah.

P: Because the creators of a specific AI tool are born in a place, they have a culture.

P: And every AI does not reply the same thing when you ask them a question.

P: So yeah, for sure.

P: Also, if you ask about the interpretation of a specific result, they don't know where you are from exactly.

P: So the scores are not the same in the US, in Spain, or in England.

P: I don't think they contrast it with the specific population, so we should be careful there.

I: Yeah.

I: Um, what concerns do you have about transparency or explainability of AI decisions?

I: So that's mostly like if AI gives you an answer or some information, what concerns are there with the fact that you don't really know where it came from.

I: It's not transparent.

I: It's not showing, like, this is my source.

I: Sometimes it just makes stuff up, right?

I: So have you thought about transparency as in showing where it came from, and explainability as in explaining how it came to that conclusion?

P: Um.

P: Okay.

P: I usually assume it's based on all the information that's on the internet.

P: I assume it's scientifically proven.

P: And that the AI trusts information from reliable sources and rules out information that's not reliable, because it doesn't follow the scientific method, for instance.

P: I hardly ever check where the information came from.

P: But in my personal or student life, if I need something for a project or my thesis and I'm not sure how it works, I ask about the specific source and then I check if it's reliable.

P: But at work, I hardly ever do that.

I: Yeah, makes sense.

I: Any other limitations or risks you can come up with now?

I: If not, that's also an answer.

P: I'm just checking because it's open.

P: Not that I can come up with now.

P: Maybe if it arises later, I'll let you know.

I: Okay.

I: This next part is about client perspective and therapeutic alliance.

I: How do you think clients respond to AI being used in their treatment?

P: I don't think they know we use it most times.

P: And they're not very familiar with it.

P: So I don't really know.

P: These are just my thoughts.

P: Maybe younger people who are more familiar with it would be okay.

P: But some people might be scared that we're not doing our job properly because we're using these kinds of tools.

P: They might think we're lazy or not professional enough.

P: Or that we don't have critical thinking.

P: But usually they don't know, and it depends on the professional.

P: I think I use it properly, only when I need it to make sure that I'm right.

P: So I understand their fears.

I: For this one I also want a score.

I: If you tell clients you want to use AI in their treatment, how willing do you think they would be, on a scale from one to ten?

P: I think that if we're transparent and tell them why we want to use it and the benefits, they would be very willing.

P: Maybe eight or nine.

P: Some of our patients are pretty old and they're okay with anything.

P: At least that's my experience.

I: Yeah.

I: Do you think clients would be more or less engaged with AI-supported treatment?

P: It depends on how I use it.

P: The way I use it, I don't think it would change their engagement.

P: Because I use it for homework.

I: Yeah.

P: It's basically the same homework.

I: Just more easily made.

I: Because in general the treatment doesn't really change, you're just making homework faster and then checking if it's in line with what you want.

I: Right.

I: Okay.

I: This is the last question.

I: This is about professional identity, autonomy, and future conditions.

I: Which aspects of your work should remain strictly human, and what would you need to feel confident about using AI safely in practice?

P: Okay.

P: What I need is to know that the data I enter into the program is safe.

P: Just in case I accidentally enter some personal information.

P: That's the top one.

P: Can you repeat the first part of the question, please?

I: Which aspects of your work should remain strictly human?

P: I don't think we should leave everything to AI and run therapy or rehab exercises strictly at home.

P: Having a person by your side motivating you.

P: Understanding how you feel and showing that we actually understand.

P: That's paramount.

P: I've seen that with some patients during my internship and now.

P: Some patients, usually because of neurological problems like a brain tumor or a stroke, are depressed or apathetic.

I: Apathy is correct.

P: Through the therapeutic alliance, the therapeutic bond, you can see improvement.

P: And I don't think AI can perform that role.

I: Yeah.

P: I don't know.

P: Also, having meetings with the family, I think that's very important.

P: The way you express yourself, the way you can transmit the knowledge you have, your expectations, that can make a difference in how people react to the treatment and to the center itself.

P: So I would say that can improve engagement.

I: Mm-hmm.

I: Yeah, so a follow-up question is why do you think these aspects need to stay human?

I: You mentioned a lot of things, like motivation, understanding emotions, and the therapeutic alliance.

I: Why should they stay human?

I: So the follow-up question I had was why do you think these aspects need to stay human, but you already mentioned quite a few things.

I: I'm summarizing: you said motivation, helping clients understand their emotions, having a better connection with family, and better results, and more engagement.

I: Yeah, yeah.

I: Okay, is there anything else you want to add?

I: Not in terms of my answers, but you have questions for me as well?

I: Yeah, okay.

I: Then I will stop the recording.

I: Yeah, yeah.

I: Okay.